

BILATERAL SYNCHRONOUS DUCTAL CARCINOMA IN SITU DETECTED ON BREAST MRI FOLLOWING IMAGING-PATHOLOGY DISCORDANCE

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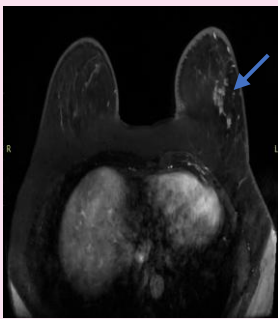
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INTRODUCTION

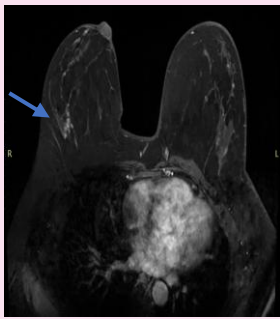
Ductal carcinoma in situ (DCIS) commonly presents as suspicious microcalcifications on mammography. However, some lesions present as non-mass enhancement (NME) on breast magnetic resonance imaging (MRI), which may be occult or underestimated on conventional imaging. MRI is valuable in evaluating disease extent and identifying additional lesions, particularly when imaging findings do not correlate with pathology. We report a case in which MRI detected occult bilateral synchronous DCIS, resulting in a significant change in surgical management.

BREAST MRI FINDINGS



LEFT BREAST

Clustered ring enhancing non mass enhancement noted predominantly at the left lower outer quadrant (LOQ) in segmental distribution



RIGHT BREAST

Clumped enhancement pattern noted at the level of the nipple posteriorly of the right breast.

CASE PRESENTATION

A 43-year-old woman presented with spontaneous left nipple discharge and a palpable left breast lump. Initial mammography and ultrasound demonstrated a BI-RADS 3 lesion, and ultrasound-guided biopsy confirmed intermediate-grade DCIS. She underwent wide local excision with sentinel lymph node biopsy followed by re-excision because of involved and close surgical margins. Histopathology subsequently demonstrated persistent intermediate- to high-grade DCIS associated with an intraductal papilloma.

PATHOLOGY & SURGICAL MANAGEMENT

Despite repeated surgery, the extent of disease remained discordant with conventional imaging findings. Breast MRI demonstrated bilateral suspicious outer quadrant non-mass enhancement. Targeted ultrasound-guided biopsies confirmed bilateral ER-positive DCIS, with intraductal papilloma containing intermediate-grade DCIS in the right breast and intermediate- to high-grade DCIS with adenosis in the left breast. Based on these findings, the planned repeat breast-conserving surgery was changed to bilateral skin-sparing mastectomy with immediate implant reconstruction.

TARGETED ULTRASOUND



Well defined hypoechoic lesion within focal ductal dilatation of the left breast at 4:00 3 cm from nipple.

DISCUSSION

This case highlights the importance of breast MRI as a problem-solving modality in patients with imaging-pathology discordance. MRI identified occult synchronous contralateral disease that was not confidently demonstrated on conventional imaging, allowing more accurate assessment of disease extent and appropriate surgical planning. Recognition of suspicious NME is essential, particularly in patients with persistent or recurrent DCIS.

CONCLUSION

Breast MRI plays a pivotal role in evaluating imaging-pathology discordance, detecting occult bilateral synchronous DCIS, and guiding multidisciplinary treatment planning. Recognition of suspicious non-mass enhancement can significantly influence surgical management and improve patient care.

REFERENCES

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